

HCI: Design Iterations

The Problem

The Finnish Student Health Service (FSHS) requires that all patients go through an onboarding process before receiving the right to make appointments for general and mental health matters via their online service. During this process, patients are required to give details on what their issue is, how long it has been happening, whether they have already received help, etc. There are two main channels for completing onboarding: over the phone or using their online chat service. The chat service reportedly has many usability problems, and both channels suffer from long wait times. When wait times are too long, it is even impossible to join the queue. These factors cause enough friction that patients may not receive the care that they need in a timely manner if at all.

User Requirements

In the second assignment, I conducted an interview with a customer of FSHS to determine what kind of usability problems they have experienced and what they expect from the onboarding process. Here are the user requirements that I was able to capture:

1. Users must be able to see an overview of the whole onboarding process from the start.
2. Users must be able to see all available contact methods.
3. Users must be able to use whichever contact method suits them best.
4. Users must be able to make an appointment without strongly authenticating.
5. Users must be able to join a queue no matter how long of a wait.
6. Users must be able to see how long the queue is (i.e., position or wait time).
7. Users must be able to receive a notification when their turn is up.
8. Users must not have to wait to see available appointment times or to know approximately when they will be seen.

Iteration 1: Survey of Diverse Low-Fidelity Designs

Goal: The goal of this design iteration is to start with a diverse assortment of low-fidelity options and narrow it down to the single most feasible direction. We need to find out which option potential users prefer the most. The focus is on the format and not the flow yet.

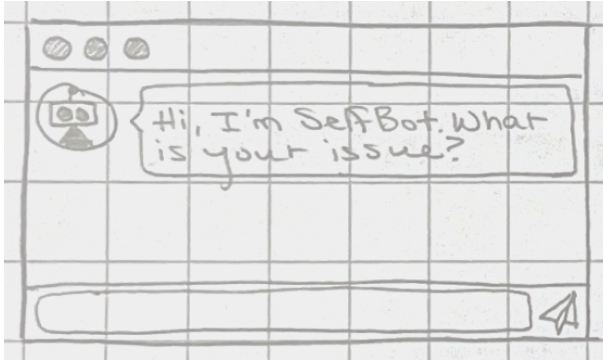
Research Questions:

1. Which contact methods do users find attractive for discussing private health issues?
2. What do users value most about their onboarding experience?

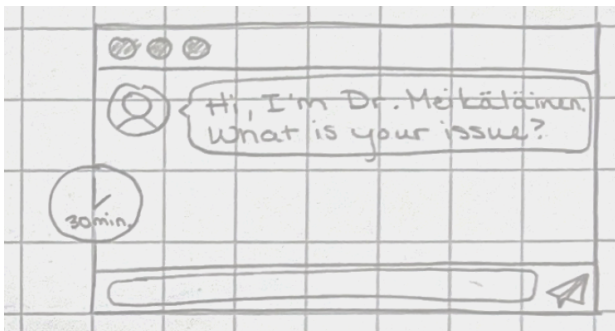
By answering these research questions, I will be able to narrow my search for the next iteration and focus on more specific details of the design. Knowing what users value most will also help me add more definition to the low-fidelity sketches for the next iteration.

Prototype: For this prototype, I created simple sketches of each proposed design. Sketches are the most appropriate technique since I wanted to create as many as possible, and the details do not matter as much as the big picture. Here are the different versions presented:

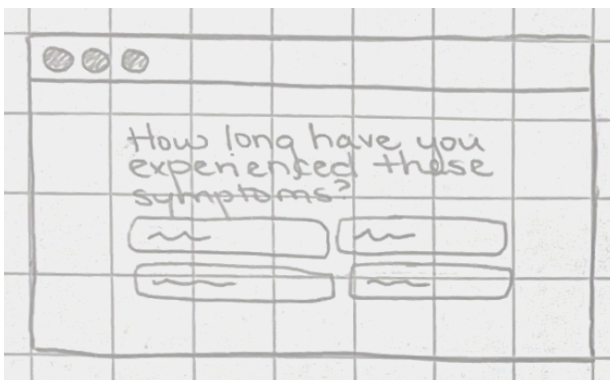
1. AI Chatbot



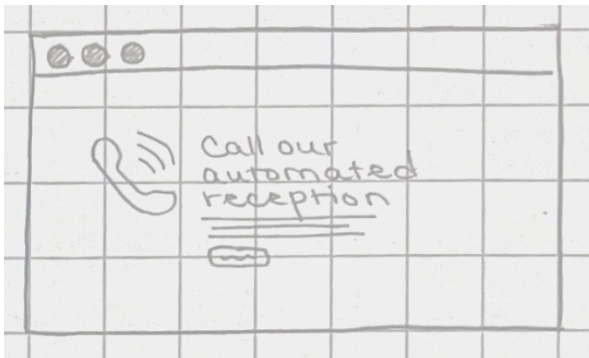
2. Human Chat with Wait



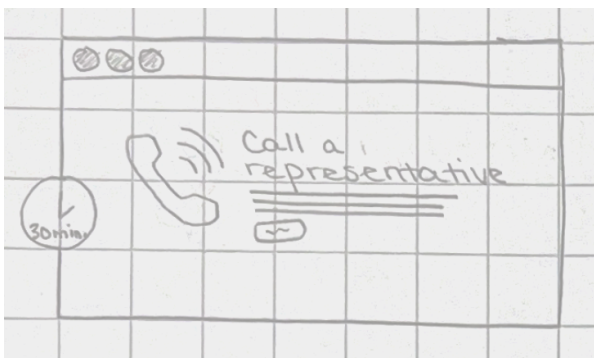
3. Guided Questionnaire



4. Automated Phone Call



5. Human Phone Call with Wait



Method: I decided to conduct a survey using Google Forms in order to elicit as many responses as possible in an easy manner. Here are the survey questions:

1. For each design, how would you rate its perceived physical effort?
 - a. Very Low Effort
 - b. Low Effort
 - c. Moderate Effort
 - d. High Effort
 - e. Very High Effort
2. For each design, how would you rate its potential for reducing pre-appointment anxiety?
 - a. Very Low Potential
 - b. Low Potential
 - c. Moderate Potential
 - d. High Potential
 - e. Very High Potential
3. Rank the designs featured above holistically.
4. Please provide a brief justification for your top-ranked design choice.
5. Please provide a brief justification for your bottom-ranked design choice.

Results: I received seven survey responses in total. I analyzed the first three questions by converting each scale to numbers and finding the average score for each design:

	Perceived Effort (↓)	Reduce Pre-appointment Anxiety (↑)	Holistic Rank (↓)
AI Chatbot	3.14	2.86	3.29
Human Chat with Wait	3.57	3.29	3.14
Guided Questionnaire	2.00	3.43	2.00
Automated Phone Call	3.57	2.71	3.29
Human Phone Call with Wait	4.00	3.29	3.29

Based on the qualitative data, the guided questionnaire was the preferred design across the board. It requires the least perceived effort, reduces pre-appointment anxiety the most, and has the highest holistic rank.

Looking at the qualitative data tells a different story (see appendix A for raw responses). The use of AI, for instance, is highly divisive. Some respondents prefer it the most, while others prefer it the least. Likewise, some respondents really value the human connection, even if it means waiting. The questionnaire is a nice middle ground. Other values that were highlighted in the free response questions include the following: accessibility, speed, convenience, information, ease, flexible input, and time to think. In order to accommodate these values, it is probably best to have multiple contact methods available. For the sake of this study, I will just continue with designing the questionnaire to start with.

Iteration 2: Guided Questionnaire Wireframe

Goal: The goal of this iteration is to determine which features of a guided questionnaire best support the user experience. In particular, we need to find out which progress indicators and input methods suit the user best.

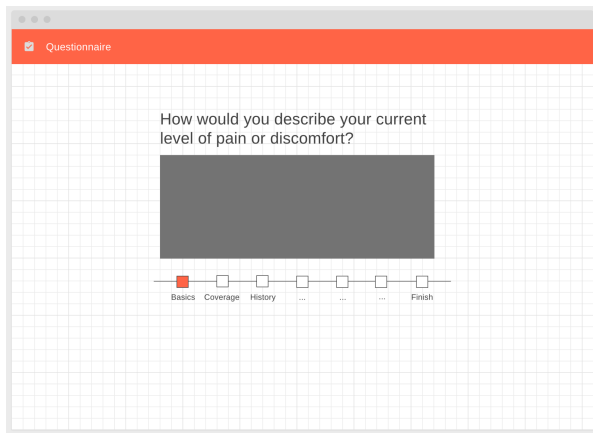
Research Questions:

1. What features of a progress indicator for users value most?
2. What features of an input method do users value most?

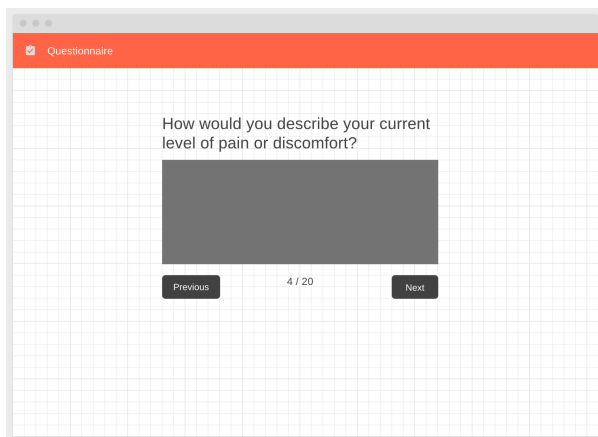
By answering these research questions, I will be ready to implement my first working version of the questionnaire. Once the proper progress indicator and input methods are in place, I can move on to designing more cosmetic features in the next iteration.

Prototype: For this prototype, I created medium-detail wireframes of each proposed design. Wireframes are the most appropriate technique since I need to be able to add enough detail (progress indicators and input methods) without wasting too much time on finishing touches. Here are the different versions presented:

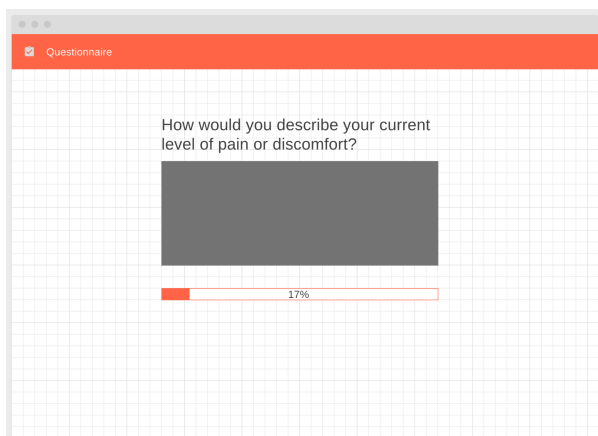
1. Chunked Progress Bar



2. Question Counter



3. Percentage Progress Bar



4. Chat Input

Questionnaire

Zombie ipsum reversus ab viral inferno, nam rick grimes malum.

De carne lumbering.

How would you describe your current level of pain or discomfort?

No pain

Mild

Moderate

Excruciating

5. Scale Input

Questionnaire

How would you describe your current level of pain or discomfort?

0 - No pain 10 - Excruciating

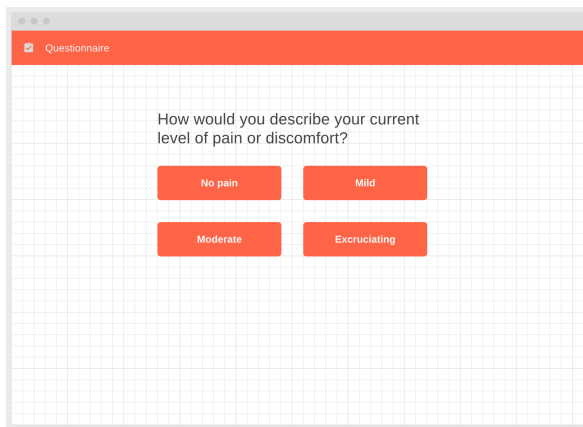
6. Text Input

Questionnaire

How would you describe your current level of pain or discomfort?

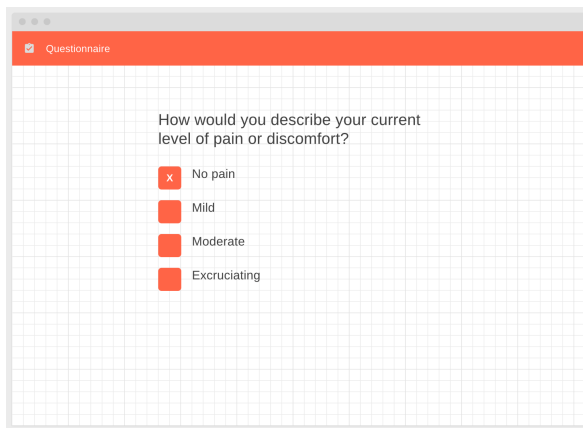
Submit

7. Button Input



The screenshot shows a web browser window with a title bar containing three small circles. The browser's address bar shows a red tab with a white icon and the text 'Questionnaire'. The main content area has a light gray grid background. At the top of the grid, the text 'How would you describe your current level of pain or discomfort?' is centered. Below this text, there are four red rectangular buttons arranged in a 2x2 grid. The buttons are labeled 'No pain', 'Mild', 'Moderate', and 'Excruciating' from top-left to bottom-right.

8. Radio Input



The screenshot shows a web browser window with a title bar containing three small circles. The browser's address bar shows a red tab with a white icon and the text 'Questionnaire'. The main content area has a light gray grid background. At the top of the grid, the text 'How would you describe your current level of pain or discomfort?' is centered. Below this text, there are four red square radio buttons arranged in a vertical column. Each radio button is followed by a label: 'No pain', 'Mild', 'Moderate', and 'Excruciating' from top to bottom. The first radio button, corresponding to 'No pain', has a small white 'x' inside it, indicating it is the selected option.

Method: I decided to conduct a semi-structured interview in order to elicit detailed feedback on the features in question. Here are the interview questions:

1. Introduce the progress indicators. For each progress indicator:
 - a. What are your first impressions of this progress indicator?
 - b. Is the progress indicator intuitive? Why or why not?
2. With all progress indicators:
 - a. Which of these indicators is your favorite? Why?
 - b. Which of these indicators is your least favorite? Why?
3. Introduce the input methods. For each input method:
 - a. What are your first impressions of this input method?
 - b. Is the input method intuitive to use? Why or why not?
4. With all input methods:
 - a. Which of these input methods is your favorite? Why?
 - b. Which of these input methods is your least favorite? Why?
5. Is there anything else you would like to share about the designs you have seen today?

Results: See Appendix B for a transcript of my interview. I learned a lot about what users value in a progress indicator and an input method. I chose to study these two components because they are the most important factors in bridging the gap of evaluation and the gap of execution.

My interviewee seemed to really value time management, and this greatly impacted their evaluation of the progress bars. They talked about a hypothetical scenario where they were filling out the questionnaire on a tram and wanted to be able to estimate where they were in the process. The percentage progress bar seemed to be the least effective at this, whereas the question counter proved to be effective. They enjoyed a one-question-per-page design since it helped them feel like they were making progress, and that their responses were being saved. Some combination of the question counter and segmented progress bar seems to be best.

Regarding input methods, my interviewee valued both speed and expression. When picking their favorite method, they were torn between the speed of the buttons and the expressiveness of the text box. They absolutely hated the chat interface because it gave the impression of an expressive environment without the opportunity to really express themselves. It also gave the impression that they were talking to a human, when they were not. The slider was slightly more expressive, but it created undue cognitive load. Again, some combination of quick response buttons with an open field to clarify their responses if necessary seems to be the optimal method.

Some other design tips also came up in our discussion that were unrelated to the research questions. First, the interviewee liked the idea of being able to save your responses and return to the questionnaire later. Second, they also desired some indicator of completion at the end of the questionnaire—an indication that a real professional will look at their responses and get back to them within an estimated amount of time. This indicator would give the patient comfort, closure, and potentially some next steps.

Iteration 3: Guided Questionnaire Mockup

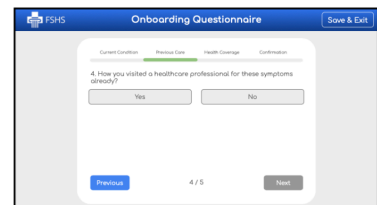
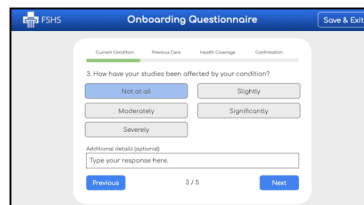
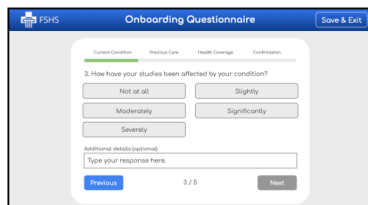
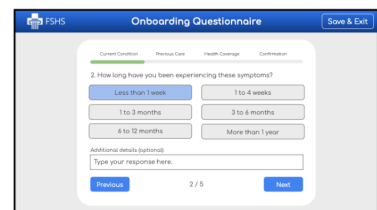
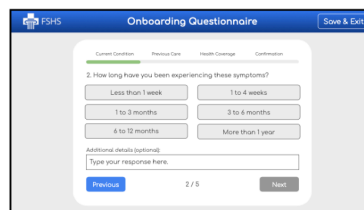
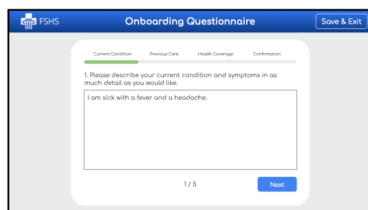
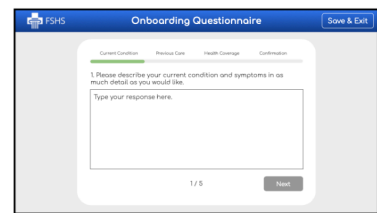
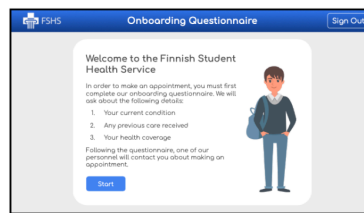
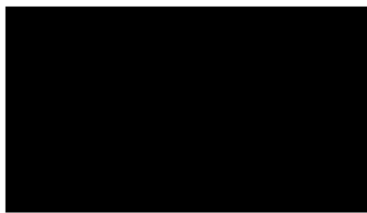
Goal: The goal of this iteration is to find any unforeseen usability problems in my first working prototype. In particular, we need to find out any points where the user is confused by the user interface or where the questionnaire is unable to capture the information adequately.

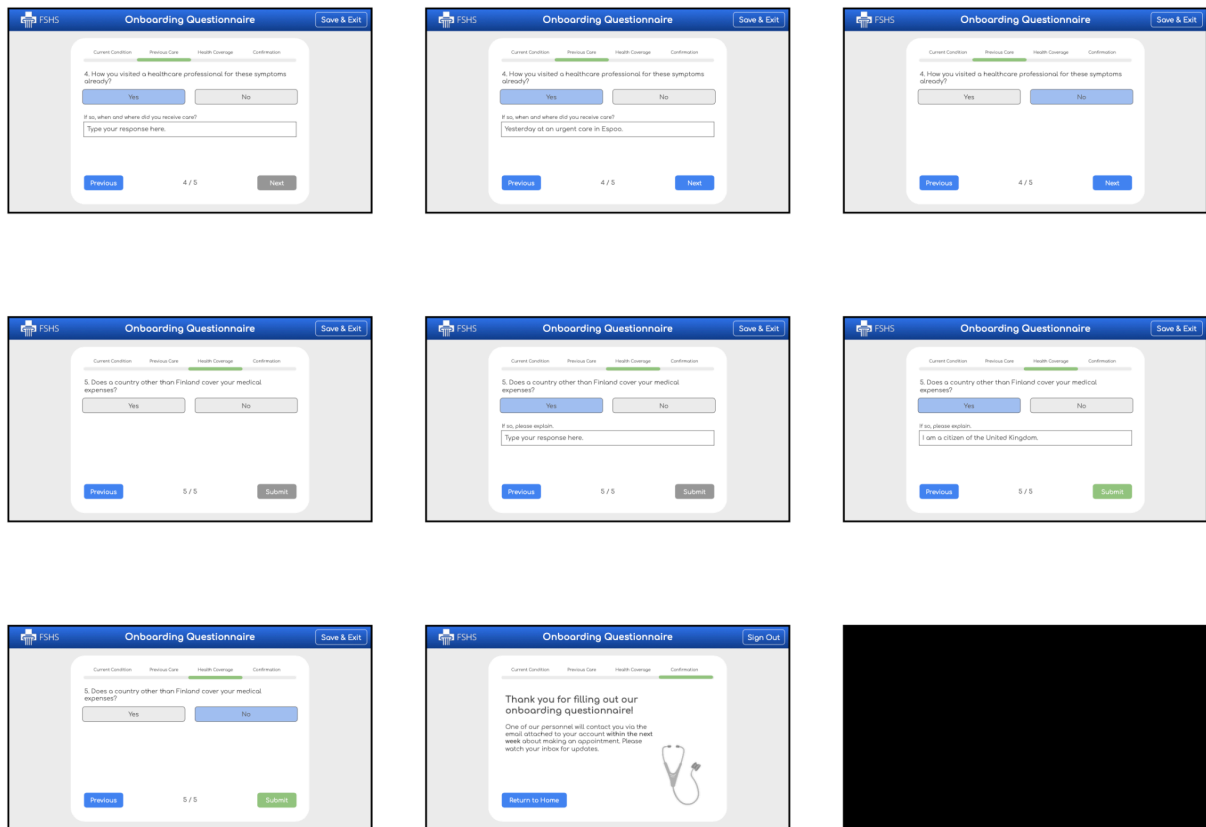
Research Questions:

1. Can users successfully identify where they are in the process and how much work is left without confusion?
2. Are there any points in the questionnaire that distract, delay, or discourage the user?

By answering these narrow research questions, I will be ready to make any (hopefully) small changes before going into production.

Prototype: For this prototype, I created high-detail mockups of each step in the questionnaire flow in order to simulate the complete experience. I used Google Slides to make the mockups due to my familiarity with the app and its ease of use. Mockups are the most appropriate technique since I am getting close to the actual implementation and want something to pass to the hypothetical developers. The experience should also be as authentic as possible for proper testing. Here are the different mockups presented:





Method: I decided to conduct a couple think-aloud cognitive walkthroughs in order to get detailed feedback on the entire user experience and to catch usability errors along the way. Here was the procedure I followed:

1. Inform the user of their persona and goal: “You are a student at the University of Helsinki. You are experiencing a headache and a fever, and you want to make an appointment with the Finnish Student Health Service, so you log on to their website. Please think aloud as you navigate the interface and work to achieve your goal of making an appointment.”
2. Show the user the first mockup and let them navigate themselves.
3. Encourage the user periodically to keep thinking aloud as they work.
4. Thank the user for their participation when they either complete the task or give up.
5. Probe the subject about any potential usability concerns.

Results: See Appendix C for a transcript of my two 10- to 20-minute cognitive walkthroughs. Overall, the participants felt like the interface was well-designed and intuitive. They were able to answer all of the questions without making any mistakes or pausing to figure out what they were supposed to do next. I analyzed the transcripts by reading through them and looking for relevant information that could help answer my initial research questions. The relevant findings will be presented below.

Regarding the first research question, the participants did not actually pay much attention to the progress indicators except to get their bearings at the beginning of the survey. The first participant expressed confusion that the number of sections along the top did not align with the number of questions along the bottom. The second participant concurred and suggested that each section's questions be consolidated onto one page so that the two progress indicators would match. This proposal somewhat conflicts with the suggestion of the participant from the semi-structured interview during the last design iteration, but it might be a nice compromise.

The first participant also requested that the introduction screen contain a short estimation for how long the whole survey will take. As a busy student, knowing whether the survey is only a few minutes or half an hour will help them effectively plan for when to take it. Since the survey is short, they remarked that the "Save & Exit" button is likely unnecessary.

Regarding the second research question, the second participant felt some discouragement upon completion of the survey. The generic message that they will receive a follow-up email "within the next week" did not seem to match their expectations and the urgency of their sickness. Perhaps if the service allows for it, the message could be revised to say "within the next business day." For extremely urgent matters, it may also be useful to provide the user with a phone number to dial to escalate their case. This again allows for human contact if that is what the user values.

The hybrid input method (selection with an optional comment box) seemed to go over really well with the two participants. The buttons made it fast and easy to select a response, and the scales provided were comprehensive enough. The textboxes enabled the users to provide nuance to their responses and helped them to feel more heard. Since they were optional, it did not slow down users who valued efficiency over detail and completeness.

I tried not to lead on the participants too much about the purpose of the study, but they ultimately were a little confused and commented about a wide variety of non-central aspects, like the content of the questions themselves. In the future I would provide a more comprehensive orientation message about the goals of our interaction so they are less confused.

Discussion

I think that my final design is very efficient and usable for the problem of patient onboarding at the Finnish Student Health Service. In my most recent iteration, the study participants

praised the platform for its ease-of-use. There is still some friction to be resolved with the actual appointment-making process, but that is beyond the scope of this study.

As discussed in the previous section, there are a few finishing touches that could be done. Namely, the two progress indicators need to be more aligned, and the introductory and confirmation copy could use a few small tweaks.

The final design is markedly more pleasant for users than the current system. The questionnaire is more broadly liked than the other proposals like the chatbot, for instance. Still, some patients may prefer to wait for human contact from the very beginning. There is still room for such an implementation to be available in parallel with this solution.

During the design process, there were times when the users confirmed my suspicions and times when they rejected them. I learned that the hard work of user research is necessary even when we think that we know how the user will behave. The user is also not some monolithic entity, as I saw in my initial survey. The responses, preferences, and values varied widely, but they did ultimately convene around the questionnaire model. Also, starting my research with broad research questions and slowly narrowing them with each iteration saved a lot of work and additional headaches. I did not have to design a fully-featured chatbot only to find out that some users abhor the idea. The user-centered design process arrives at systems that are broadly usable.

Appendix A: Survey Free-response Questions

Please provide a brief justification for your top-ranked design choice.

1. I feel like texting with a person reduces stress for people who don't like to call and it's convenient for everyone (i have experienced this way of assistance elsewhere and it was more helpful and less stressing than a call)
2. The AI bot is easily accessible and can help to find information quickly.
3. A questionnaire is low effort, lets the user either be quick or have time to think about their replies, and makes it feel like the process is progressing
4. questionnaires let me answer at my own pace
5. Really easy and low effort to fill and no wait
6. I prefer to chat with a real person, and I can use the chat function anywhere I am, including school
7. With a guided questionnaire I have all the time and options to tell what I think I need to tell, but also what they need to know.

Please provide a brief justification for your bottom-ranked design choice.

1. I hate AI chatboxes because they're trained to answer very specific questions and most of the time are not replying to anything I ask if it's not prompted correctly
2. The human phone call with wait takes up a long time, but can probably provide the best info on my issue. I would probably have a little chat with the AI-bot to see if it can give me ideas that I want to get more info on during the phone call.
3. AI chatbots often feel like they just talk in circles and prevent you from progressing with the request, so they immediately trigger frustration in me
4. talking to humans is scary
5. You will have to write instead of just clicking options or speaking. In addition the waiting time is inconvenient.
6. The automated phone call feels inhumane and stressful, the AI often misunderstands
7. I expect to not be able to tell all I need on the phone or I would accidentally leave out information for them

Appendix B: Wireframe Interview Transcript

A: Uh, okay. I'm going to show you a series of progress indicators- Okay. ... for a questionnaire to be administered at the, uh, during the patient onboarding process- Okay. ... at the Finnish Student Health Services. Are you familiar with the Finnish Student Health Services?

B: I am. (laughs)

A: Good. Um, what are your first impressions of this progress indicator here?

B: Uh, it doesn't really tell me much. It's kinda vague.

A: Okay. You think it's vague?

B: I would say it's kinda vague.

A: Um, is it intuitive?

B: Yeah. Yeah.

A: It is?

B: But- Maybe like the, the word choices or what kinda tip me off, like I'm not really sure what that's supposed to mean. But like the, the, the shape itself is, you know, logical. So, it's intuitive because- It's linear.

A: ... it's linear, and you view time as a linear construct. (laughs)

B: Sure. (laughs)

A: Um, as a side note, in this gray box would go like the input method of how you respond to the question. Oh, okay. So, that's not part of it. All right. This is just like a sample question. And then this is the focus of, of our- Mm-hmm. Okay. ... of our discussion.

B: All right.

A: Um, so you said that it doesn't tell you much-

B: Yeah.

A: ... but it's intuitive?

B: Yeah. Like the shape itself is intuitive, like I, I do enjoy a good linear, uh, progress indicator. Mm-hmm. But like the basics, coverage, history, so on, it doesn't really tell me much.

A: The, so the labels- But if, if there were better labels, would it be more intuitive?

B: I would say so, yeah. Okay. Yeah. Like, uh, granted that this is the actual like realistic number of questions. Uh- Like if this is just-

A: These are representative of sections, not individual questions.

B: Okay, yeah. Yeah. Yeah.

A: That's better?

B: Yeah.

A: That it's not individual questions?

B: No, like I don't really care, but like, um, it's nice to see... Wait, my brain isn't working. Um, it's nice to be able to like view at a glance the entire like process. So, they don't necessarily need to be numbered, but as long as I can see that every box is actually being checked as I do my things and there's not like a, you know, like a secret hidden section of like five more boxes- Yeah. ... that's like missing- It's the entire-

A: Yeah, that's the whole timeline.

B: ... the entire timeline. That's the most important to me.

A: Okay. Yeah. Uh, this is a percentage progress bar. What are your first impressions of this progress indicator?

B: It's all right. Um, I guess my only gripe with these is, uh, I'm, I'm, uh, I assume that it only shows you like the... Can, can a little percentage is based on the page which you're on, but it necessar- doesn't necessarily tell you how long the page is or how many questions the page has or how much time it takes to answer them. So, if it says, oh, you're like 90% done, but then it's like seven really hard and long open-ended questions, this doesn't really tell me much. But like as a concept, I'm not opposed to this.

A: So you care about how much time it will take to finish the survey?

B: Yeah.

A: And maybe mental effort?

B: Yeah.

A: Like, do I have to write something?

B: Sure, yeah. Um... So like if, for example, if every question was on a page of its own, then this would make more sense.

A: Every question is on a page of its own.

B: Yeah. Then I would say it's like pretty okay.

A: Unless would you prefer- (laughs) ... to have all of the questions on one scrollable page?

B: Not really, no. 'Cause that also, um, that also kind of, uh, makes me think if like, um, if like the progress is gonna be lost if I close the page. So if I have kind of like pages that I transfer between- Mm-hmm. ... that g- that give me the option to kind of save my progress and continue later- Yeah. ... or if I can like view at a glance, uh, how much time I have and how much time it's gonna take me, then that's nicer because if I'm doing it like in a tram or something- Yeah. ... on the tram, then it's good to know kind of where you're at and if you're gonna be able to stop mid-way or not, or like pause mid-way or not. That makes sense.

A: Yeah. So you would say this is, is this indicator intuitive?

B: Maybe I- Hm, I don't know. It depends, like if every question is on its own page, then it's more intuitive. But like gen- generally, because it has a kind of like, has to have a disclaimer, maybe it's less intuitive for me.

A: Okay. Yeah. Uh, this is a question number progress indicator. What is your first impression of-

B: I enjoy this one.

A: You enjoy this one?

B: Yeah. (laughs)

A: Why do you enjoy this one?

B: I enjoy a good delimited number. (laughs)

A: (laughs) Okay.

B: Uh, I like to know where I'm at. And also just like, I don't know if it's true, but it like indicates that I can kind of like pause and like maybe like kind of lock my phone and then kind of still remember which question I'm on. You know? Like I can like put my phone in my pocket, run into the tram, and then continue, and I can be like, "Oh, I was on question four." You know? Okay. So- And I only have this much. Oh, okay, I only, I only have two questions

left. I have like five minutes left till I have to leave or whatever. Mm-hmm. So it's like, yeah.

I like structure. (laughs)

A: You like this one. Oh, you like structure? Uh, would you say it's intuitive?

B: Yes.

A: Yes. Why?

B: 'Cause you can tell how much four is out of 20 (laughs) pretty easily.

A: Yeah, but, um, does it matter then that you don't necessarily know if questions... what's up- that certain questions will take longer than others? I mean- (coughs)...

B: I know that, like, that's probably true that, like, it doesn't really... you can't deduce that from this, like you can't deduce the time that you're gonna need based on this, but I still somehow like this one- Mm-hmm. ... as well.

A: Yeah. Okay. I'm going to open them all now so you can see them all at once. Yeah. It did not work. Eh. Okay. Uh, which of these indicators is your favorite?

B: I mean, now it's tough because I do like to... like, I, I enjoy the extra information here, if it has, like, kind of themes. Mm, I don't know. Like, I actually, like, I know that this is contrary to what I just said, but I do kinda gravitate towards the linear visual whatever indicator.

A: The segmented progress bar?

B: Yeah, exactly. But if it was, like, kind of more clearly... no, actually, I don't know. Ah. I'm honestly not sure. I'm torn between the, this one and this one.

A: The numbered questions- And the... Yeah. ... and the segmented progress bar. Why?

B: Um, because I would like... like, the, the numbered one doesn't give me any information on, like, the themes and, like, the- Mm-hmm. ... what the questions are gonna be like. Um, but the linear one, I guess if it's themes, you can't really tell if a theme has, like, two or 20 questions.

A: Yeah.

B: So yeah, I would, I would say I prefer knowing the total number of questions, so maybe the numbered one if I had to choose.

A: Okay. Yeah. Um, we will now move on to, uh, input methods. So this is, uh, buttons?

B: Mm-hmm.

A: What are your first impressions of this input method, the buttons?

B: Sure. (laughs)

A: Sure?

B: Sure.

A: Okay. Why sure?

B: Um, because it does its job, but I guess there must be instances where, like, four options wouldn't be adequate enough. Yeah. Or, like, any amount of options. Mm-hmm. So, like, there's always, like... I'm fine with this as long as there's, like, kind of, like, final, um, option to also have an open-ended answer.

A: Okay. Two questions. Uh, would you say this is intuitive to use?

B: Yeah.

A: Why?

B: Because it's easy. Yeah.

A: Yeah. Okay. Uh, okay. This is the chat-like input method. Uh, what are your first impressions of this input method?

B: My first impression is that I hate it, but I can't give you any reason why. (laughs) I guess just because usually these just, like, kind of go in circles and then you're just like, "Please, can you guide me to a human?" And it's like, "Thank you for your feedback."

A: Um- So, this isn't actually an open-ended chat. It's effectively the same as a questionnaire. Because these are your options.

B: Yeah. I mean, I hate it because it gives me the impression that it's a chat, but then it doesn't work like a chat. So, like, it's less intuitive because it makes me wanna chat. (laughs)

A: You, you wish... by seeing a, a chat interface makes you want to have open, like, an open-... open input method?

B: Yes. Like, I would prefer it to look like, um, like, just options instead of looking like a chat but not being a chat.

A: Okay. Um, would you say this method is intuitive to use?

B: No. (laughs)

A: Why not?

B: Because it's lies. (laughs)

A: Okay. That is true. Yeah. Uh, this is the radio button input method.

B: The what button?

A: They're called radio buttons. Um, what is your first impression of this?

B: I would say it's, um, equally put, uh, equally intuitive to the first one, so to the-

A: To the buttons.

B: ... the buttons, yeah.

A: Um, would you say it's intuitive?

B: Yeah.

A: Um, does either the buttons or the radio buttons have an advantage?

B: Um, I'm mostly not sure. This kinda makes me feel like there shou- like, there might be a possibility to choose multiple, which probably doesn't exist. Mm-hmm. But, yeah, no. Okay. They're pretty much the same to me.

A: Uh, this is the scale input method. What are your first impressions of this input method?

B: It's all right. Um, I feel like it would maybe benefit from a couple, like, kind of checkpoints with more- Intermediate labels. ... described labels, yeah. Yeah. Because, uh, no pain and excruciating (laughs)- (laughs) ... uh, there's a lot of things in between, uh, whenever I've had to, uh, use those, I found it kind of difficult to, like, save my pain to, like, a seven or a five, but- (laughs) Yeah, I don't know. Maybe if it had more labels, it would be nice.

A: Okay. Would you say it's intuitive to use?

B: It is intuitive in the way that like, it's obvious what you're supposed to do. Yeah. But maybe it's just like a little bit difficult. Like, like maybe it's easier for, to choose an answer from a limited amount of options than to like- Yeah. ... evaluate if you're feeling a seven or an eight.

A: Yeah. That makes sense. Yeah. Um, I will now show you... Or I guess there's one more.

This is a text box. Mm. Uh, what is your first impression of this input method?

B: Uh, it's nice. Mm, yeah. Like, I don't know. This, this kind of prompts for like a lengthier description of your symptom set. Or I would assume that I'm supposed to actually like kind of describe the situation or like-

A: Um, don't focus too much on the, the question in particular.

B: Yeah. But like in general, like the presence of a text box make, makes me think okay, think okay, this is the time for me to actually like describe my situation in a lengthier way. Yeah.

A: Um, so yeah. That's your impression?

B: I guess that's my impression, yeah.

A: Do you think it's intuitive to use?

B: Yeah.

A: You've seen text boxes in real life?

B: I have seen a text box before too.

A: Yeah, that makes sense. Um, okay, now I will open all of them.

B: Are you actually recording?

A: Yeah. (laughs)

B: Just checking.

A: That would be tragic if I wasn't. Yeah. (laughs) Okay. Um, there are five in this, so I don't know how I'm going to fit all of them.

B: You can teach me how to do this later.

A: Yeah. Uh, let's see which one do I not have. But, yeah. Okay, uh, you can, you can kind of see them all.

B: I can make it out.

A: Yeah. Um, which of these input methods is your favorite?

B: Hmm, honestly, I guess the button.

A: The buttons?

B: Yeah.

A: These?

B: Yeah.

A: Why?

B: 'Cause it's very like intuitive and easy. But again, like I would be frustrated if that was the only option. So, I would like to have an open-ended question at some point. But like I feel like this is the easiest way to just like kind of, yeah, I don't know, assess the situation.

A: Quickly answer?

B: Yeah. Like if this kind of actually affects, um, what the site is doing or like how the doctors are gonna like... how quickly the doctors are gonna respond to me- Yeah. ... this is like the most obvious option, I would say.

A: Mm-hmm. Uh, which is your least favorite?

B: Hmm, the chat. (laughs)

A: Why? (coughs)

B: Uh, because I feel like it's an unintuitive and like kind of like an unnecessary way to organize the buttons, 'cause you, you could just like- Yeah. It's a, it's, it's a- You could just like present them as buttons. It's effectively the same as the buttons, but it's misleading. Yeah. 'Cause there's no human on the other side.

A: Yeah. Or robot.

B: Or an open-ended question- Yeah.

A: Yeah. Um, you mentioned liking open-ended input at some point. Um, would you like combining the buttons with open-ended input?

B: Yes.

A: So that there's always an opportunity to write in?

B: Yeah. Because again, like the, the scale between moderate and excruciating pain is quite large.

A: So- There should have been a severe-

B: Yeah. ... perhaps in the, in between those. Yeah. But like any kind of arbitrary number that they're gonna come up with, there's always nuance in that. Yeah. So, um, I think especially in like medical situations, you should always be allowed to kind of explain it- Yeah. ... openly in some way or like in an open-ended question.

A: Yeah. So buttons are good for quick responses. And text boxes are good to provide more context.

B: Yeah.

A: Um, is there any other... Do you have any other thoughts about these designs, both the input methods and the progress indicators that you wish to share?

B: Hmm, I don't know. I guess like the point, like the main point that I would like the website to message to me is that my... hmm, that what I, what I answer actually makes a difference. And like that the progress is actually progressing. (laughs) And that like if I do something on the web page, I'm actually going somewhere. Yeah. Which is like kind of what the AI chatbots usually don't feel like. Like it just feels like it's gonna circle me, me back to, "Okay, describe me your symptoms again," without me ever being able to kind of like send anything to an actual human professional. So like if I have like a limited set of questions and there's like a submit, and then there's like a feedback saying, "Okay, we, we will now evaluate your symptoms." Like- Mm-hmm. ... I just want... I like to feel like, um, like I'm being heard eventually. Like what I did here in the questionnaire-

A: Like you're not jumping through hoops.

B: ... did something. Yeah, exactly. Yeah. That I achieved something now and now somebody, a professional is actually evaluating my case. Okay. After I submit it.

A: Um, would you like then some kind of an... a wait time estimate? Or like when you finish the questionnaire- ... some kind of indicator of when you'll hear back?

B: That would be nice. Yeah.

A: Or are you okay with it just being like, "We'll get back to you eventually."

B: I guess I'm, I guess I'm okay with, "We'll get back to you," because that's usually the case. Yeah. But it would be nice to have like, at least like, "Okay, we'll reply in a month," is better than nothing.

A: Yeah. Yeah. Okay.

B: Yeah. It doesn't have to be like, um... Like it doesn't have to be- Real-time. ... updated in real time or anything. Yeah. But just like, "Okay, generally we reply in like four days." Within a month. Yeah.

A: Okay. Yeah. Uh, thank you for your input.

B: You're very welcome.

A: You are a valued participant.

B: (laughs) Appreciate that.

Appendix C: Mockup Think-aloud Interview Transcripts

Participant 1

Speaker 1

Okay. Um, you are a student at the University of Helsinki. You are experiencing a headache and a fever and you want to make an appointment with the Finnish Student Health Service so you log onto their website. Please think aloud as you navigate the interface and work to achieve your goal of making an appointment.

Speaker 2

Okay.

Speaker 1

I guess I should say this is a hypothetical.

Speaker 2

Yes.

Speaker 1

Obviously.

Speaker 2

Yeah.

Speaker 1

And, uh, I will just be observing, but remember to think aloud.

Speaker 2

Okay.

Speaker 1

And I might remind you if you trail off.

Speaker 2

Okay. All right. Looks like a little intro to make sure I know what I'm doing here. "First, complete your onboard questionnaire. We will ask about the following details."

Speaker 1

And then just tell me what you would do.

Speaker 2

Okay.

Speaker 1

Don't actually interact with it.

Speaker 2

Oh, you don't want me to interact with it?

Speaker 1

No.

Speaker 2

Okay. So... Yeah, they're gonna want to know my details, as they should, and it's kind of letting me know there's gonna be a questionnaire and I click start.

Speaker 1

Okay.

Speaker 2

Pretty straightforward. Uh, "Describe your current condition and symptoms in as much detail..." That makes sense. I would fill out how I'm feeling. Like-

Speaker 1

How would you do that?

Speaker 2

I would type it into the text box there.

Speaker 1

How would you type into it? (laughs)

Speaker 2

I would click-

Speaker 1

Well...

Speaker 2

... click in and then type.

Speaker 1

You would click into it and then-

Speaker 2

Click in and then type. I would assume... There you go. And now that I've typed in there, uh, I can see that it says, "One of five," so it looks like there's five steps to this even though across the top it looks like there's four. But, uh, I guess the first, the first one was the intro page, I'm guessing. Okay, so then I would click Next. "How long have you been experiencing the symptom?" So giving an (clears throat), uh, detail for how long I've been dealing with this condition. Um, has a breakdown of the times. I can put in some additional details and click Next, uh, after I click one of the, one of the, uh, boxes and potentially typing in additional details.

Speaker 1

So what do you wanna do?

Speaker 2

So let's say I've been experiencing it less than a week.

Speaker 1

So-

Speaker 2

And then I'm gonna type in additional-

Speaker 1

So w- so what do you do? (laughs)

Speaker 2

So I would cli- I would move the mouse-

Speaker 1

(laughs)

Speaker 2

... and I would click less than a week. And then I would move the mouse down to Next because I don't have any additional details to provide. Um, "How have your studies been affected by your condition?" I would, uh, move the mouse to Slightly and click. Um, or Not at all, and click Next. And, "Have you visited a healthcare professional for these symptoms already?" I would click No because this is my first, uh, first go around here. I can see I'm four steps of five steps in. Click Next. Uh, "Does a country other than Finland cover your medical expenses?" I would click No because I'm student and then I would say Submit.

Speaker 1

You would say Submit?

Speaker 2

That would click Submit.

Speaker 1

(laughs)

Speaker 2

Click Submit. And it lets me know that it was submitted. I filled it out successfully and it lets me know, uh, what to expect next, which is within the next week we'll be contacted about making an appointment. And I will see that in my email inbox. And I can click Return to Home if I wanted to get out of the questionnaire.

Speaker 1

Okay. So, uh, what did you think about, uh, the navigation of this questionnaire?

Speaker 2

Uh, really straightforward. I like the wizard approach. Uh, it seems like it was gathering the pertinent information for me, the information that I needed for, uh, letting them know what they need to know to get treatment.

Speaker 1

Um, you mentioned that you were confused why on the top it looked like there are four steps-

Speaker 2

Yeah.

Speaker 1

... and on the bottom it looks like there are five steps.

Speaker 2

That's right. I'm guessing the first step-

Speaker 1

Is that problematic?

Speaker 2

Uh, no. Was the fifth step just the confirmation or the Return Home?

Speaker 1

So the numbers correspond to questions.

Speaker 2

Yeah.

Speaker 1

And the top refers to sections.

Speaker 2

I see. I see.

Speaker 1

So here we're on the third question and it's still about-

Speaker 2

Of the current condition.

Speaker 1

... your current condition.

Speaker 2

Understood. Understood.

Speaker 1

And then the fourth question's about previous care.

Speaker 2

I see. Okay.

Speaker 1

Fifth question is about health coverage.

Speaker 2

And then the confirmation.

Speaker 1

And then the confirmation.

Speaker 2

Understood. Okay.

Speaker 1

Um, but it, like-

Speaker 2

Yeah. No. It-

Speaker 1

... that's confusing?

Speaker 2

It was not confu- it was not confusing. Yeah.

Speaker 1

Did you find having both was helpful?

Speaker 2

Uh, yeah.

Speaker 1

Even if...

Speaker 2

Yeah.

Speaker 1

Did you, um... Did you read the section titles at all?

Speaker 2

Uh, not as it was going. I read 'em at the start. I didn't read 'em as it was going along.

Speaker 1

Mm-hmm.

Speaker 2

Um, I was more focused on the fact that it was a wizard with, uh, five pages of questions, is what it seemed like.

Speaker 1

Yeah.

Speaker 2

... and didn't notice how the, the navigation across the top was changing as I went along.

Speaker 1

You didn't?

Speaker 2

Um, but I, I didn't notice that as it was going, uh, along.

Speaker 1

Did not?

Speaker 2

I didn't notice that, no.

Speaker 1

Okay.

Speaker 2

Did not notice that.

Speaker 1

Um...

Speaker 2

But it's, it's handy to know, "Hey, here's the things that I'm gonna be questioned about; current condition, previous care, health coverage and confirmation."

Speaker 1

Mm-hmm.

Speaker 2

Makes sense. And, the five steps in the, in the wizard was, was helpful to know it's, it's not long.

Speaker 1

Yeah. So, did you expect, uh, that, uh, all the questions would be about the same effort then as the first one?

Speaker 2

Yeah. Yeah. And, I, I like questionnaires that are not long, huge lists of exhaustive questions. This got right to, uh-

Speaker 1

Yeah. This is, um ... this is a proof-of-concept so-

Speaker 2

Yeah.

Speaker 1

... it would potentially be longer, but-

Speaker 2

Yeah.

Speaker 1

... but the questions are from actual, from, from the current ex- onboarding experience.

Speaker 2

Yeah.

Speaker 1

So, it is reflective of the types of questions they would ask.

Speaker 2

Yeah.

Speaker 1

Um. Is there anything that could make navigation or-

Speaker 2

Hmm.

Speaker 1

... like, progress indication or, um, I guess I should say, is it (laughs) separate those out?

Speaker 2

Yeah.

Speaker 1

Um, is there anything that would make, uh, like a progress indicator more intuitive?

Speaker 2

Uh, I don't think so. I think, uh, maybe letting know a- approximately how long it's gonna take. Like, it's really short, like three to five minutes or-

Speaker 1

Hmm.

Speaker 2

... one to three minutes. Maybe it did-

Speaker 1

S-

Speaker 2

... actually ask that.

Speaker 1

So-

Speaker 2

Yeah.

Speaker 1

... on like the start page?

Speaker 2

Right. Right.

Speaker 1

Just some estimate of how long it will take?

Speaker 2

Yeah. Yeah. 'Cause like whe- any time I'm filling out a big questionnaire, I'm like, "Do I even wanna do this?" Because-

Speaker 1

Right now, yeah.

Speaker 2

... it's 15 minutes or it's 30 minutes. Who knows how long it is? So...

Speaker 1

Would you also like to know if, that, you can save and exit? (laughs) Like-

Speaker 2

Yeah.

Speaker 1

... did you notice that you could save and exit?

Speaker 2

I s- I saw that along the way, but, uh, I didn't give it much thought.

Speaker 1

(laughs) But, would it be helpful to know at the beginning that-

Speaker 2

That, that you actually-

Speaker 1

... you, you can save and exit?

Speaker 2

Yeah. If it, uh, in particular if it was, if it said it was gonna be potentially long-

Speaker 1

Mm-hmm.

Speaker 2

... and I knew I could save and exit, that would-

Speaker 1

Yeah.

Speaker 2

... be useful information. If I saw that it was one to three minutes, I wouldn't even care if it-

Speaker 1

Yeah.

Speaker 2

... save and exit.

Speaker 1

And it would probably be short enough that it is one to three minutes.

Speaker 2

Yeah. Right.

Speaker 1

Um. Then, i- did you feel that the input methods were flexible enough for-

Speaker 2

Yeah.

Speaker 1

... for your needs?

Speaker 2

Yeah. I liked the, I liked the, the approach there.

Speaker 1

Um-

Speaker 2

That's probably how I would have done it myself.

Speaker 1

So you were never confused by-

Speaker 2

Not at all.

Speaker 1

... by input and-

Speaker 2

Nope.

Speaker 1

... you never felt like you wanted to say something that it could not-

Speaker 2

No.

Speaker 1

... collect? Yeah.

Speaker 2

Yeah.

Speaker 1

Um. Any other comments or feedback?

Speaker 2

Uh...

Speaker 1

(coughs)

Speaker 2

No, it seemed nice and, uh, clean. You know?

Speaker 1

Mm-hmm.

Speaker 2

And not, not busy and, you know, each page was, was very simple. And, I like that.

Speaker 1

Um. Do you feel like including confirmation on the-

Speaker 2

Yeah.

Speaker 1

... section bar is confusing or-

Speaker 2

Uh, no.

Speaker 1

... helpful?

Speaker 2

Helpful.

Speaker 1

Okay.

Speaker 2

Yeah. Like, I'm getting towards the end. That's like, I'm finished.

Speaker 1

Right. Yeah. It's-

Speaker 2

Yeah.

Speaker 1

... the feeling of being finished.

Speaker 2

Yep. Yep.

Speaker 1

(sighs) Uh, all right. I think that concludes our interview. Thank you for participating.

Speaker 2

You're welcome. Thanks for including me.

Participant 2

Speaker 1

(humming) All right. Uh, we're going to perform a usability study for a potential system to onboard patients at the Finnish Student Health Service. Uh, imagine that you are a student at the University of Helsinki. You are experiencing a headache and a fever (laughs), and you want to make an appointment with the Finnish Student Health Service, so you log on to their website. Please think aloud as you navigate the interface and work to achieve your goal of making an appointment. And, uh, in this scenario, you will tell me what you would do. So you would say, like, "Click on this."

Speaker 2

So I don't actually-

Speaker 1

Or...

Speaker 2

I don't actually do anything? Oh, okay.

Speaker 1

You don't actually interact, you just tell me what you would do.

Speaker 2

Okay.

Speaker 1

So, here we go. Think aloud, remember.

Speaker 2

Oh. And talk, like I'm reading it aloud? Onboarding questionnaire.

Speaker 1

Uh, you could read aloud, or-

Speaker 2

Welcome to the Finnish Student Health Service. In order to make an appointment, you must first complete the onboarding questionnaire. We will ask you the following details; your current condition, your previous care, your health coverage. Follow the questionnaire. One of our personnel will contact you about making an appointment.

Speaker 1

Make sure you speak loud enough to be-

Speaker 2

Oh.

Speaker 1

... heard.

Speaker 2

Okay. But if I'm thinking out loud, I can... That's what I'm gonna do is read it, you know?

Speaker 1

Yeah, but just, if that's, if you were gonna read it in your head (laughs).

Speaker 2

Mm-hmm.

Speaker 1

Don't think about it too much (laughs).

Speaker 2

Okay.

Speaker 1

But just speak loud enough that the microphone will hear you.

Speaker 2

Oh. I think it looks good. Sharp looking. Okay.

Speaker 1

Was... Is that what you would think?

Speaker 2

Yeah. I just thought, "Wow, that looks good."

Speaker 1

Okay.

Speaker 2

"I like the look of this." Okay. Now do I press start?

Speaker 1

You tell me what you would do.

Speaker 2

Oh. I would just then go, onboarding questionnaire s- not sign out, so start.

Speaker 1

You would click on start.

Speaker 2

Yeah. And then I would go, current condition, previous care, health coverage, confirm.

Speaker 1

Speak louder.

Speaker 2

Oh. Current condition, previous care, health coverage, confirm. Please describe your current condition and symptoms as in much detail as you would like. So, I would probably type, um, headache, however long I've had the headache, fever, what my fever is, and do next.

Speaker 1

So would you click in the box to type?

Speaker 2

I would touch the box next, or I would... Oh, would I click in the box?

Speaker 1

Yeah. Yeah.

Speaker 2

I would probab- You mean would I start typing or would I click it?

Speaker 1

Yeah.

Speaker 2

Um... I would, I would feel like I needed to click into-

Speaker 1

This isn't touch screen, so you don't-

Speaker 2

Oh.

Speaker 1

... need to (laughs)...

Speaker 2

So, okay. So then I would just type in my response.

Speaker 1

But would you click it? It's a website.

Speaker 2

Like with a mouse click?

Speaker 1

Yeah.

Speaker 2

Oh, yes. Click, yes. I would click into the square, type the response, and then I would hit next.

Speaker 1

You would click next.

Speaker 2

Yeah.

Speaker 1

Yeah.

Speaker 2

How long have you been experiencing these symptoms? So I would choose a thing, um, hopefully less than a week. Additional details, type your response here.

Speaker 1

Make sure to talk loud now.

Speaker 2

Um, so I would probably type in if I'd been taking Advil or what I've ever, whatever I've been taking or if, um... Yeah. And then I would do, I would click into next.

Speaker 1

So you would click...

Speaker 2

Oh, I would click less than a week or whatever it is.

Speaker 1

Less than a week.

Speaker 2

And then I would type what I've done probably.

Speaker 1

And then you would, uh, in here you would type that you-

Speaker 2

Additional details. Like maybe that I'm still experiencing it even though I'm taking Advil or whatever.

Speaker 1

Okay.

Speaker 2

Click next. And then how have your studies been affected by your condition? Um, out of all slightly, moderately, significantly, severely. Those are good choices. That's a good range of choices, so I would click whatever choice. Hopefully not at all, but slightly. Um, additional details. Um, maybe then I'd be able to put if I missed a test or wasn't able to read an, uh, you know...

Speaker 1

Yeah.

Speaker 2

If it hurt to read and I couldn't test or something, I would maybe type that in there and do next. How... You've... You mean have?

Speaker 1

Have. Have.

Speaker 2

So that's a mistake right there. Have you visited a healthcare professional for these symptoms already? Um, probably no, 'cause I'm showing up to the health place now. And then I would click next. Does a country other than Finland cover your medical expenses? Probably yes. And then submit. Oh, if so, US family health insurance or whatever plan. Type in my, I would type in my other medical plan.

Speaker 1

Mm-hmm.

Speaker 2

Um, and then submit. Onboarding questionnaire.

Speaker 1

What are you thinking? (laughs)

Speaker 2

Oh, I'm thinking, thanks for filling out our questionnaire. One of the personal contact TV or the email attached to your account within the next week. I'd probably think that's a long time if I have a headache and a fever. About making an appointment, please watch your inbox for updates. So, I guess if this is the system, I just think, "Okay, like, I'm at least in the queue." I said I have a headache, it's affecting my homework. I did- I've had it this long, I've taken

whatever meds and then... Yeah. I would think that's probably good. I'm expecting, um, an email back.

Speaker 1

Mm-hmm.

Speaker 2

Okay.

Speaker 1

And then you would...

Speaker 2

Then I would return home. There's nothing else I can do-

Speaker 1

Yeah.

Speaker 2

... at that point, right?

Speaker 1

Yeah.

Speaker 2

Yeah.

Speaker 1

Okay. So, um...Is this like enough details here at the end then for you to feel good that like you did your part and someone will contact you?

Speaker 2

Is there-

Speaker 1

Do you trust that or?

Speaker 2

Um, I would probably trust that but I would also wo- wonder is there another ... is there something to expedite it or get a call from somebody? Or is that the quickest ... is that what you're wanting to know? Like I would wonder if I could speak to someone sooner than within the next week by email.

Speaker 1

Yeah.

Speaker 2

So I would ... that's ... I would be hopeful to get a call back from a nurse or something.

Speaker 1

Um.

Speaker 2

So like could I leave my phone number?

Speaker 1

Meaning you would, you would prefer a call-

Speaker 2

Mm-hmm.

Speaker 1

... over an email or is that just because you think a call would be faster?

Speaker 2

Well, a call would be then I'm talking to the person to exchange, get a little Q & A if needed. An email, they're gonna email me. When I email back at th- to them, are they gonna respond back to me? You know, an email would be immediate conversation. I mean, a phone call

would be conversation. An email could be more delay if I don't see it immediately and then if I qu-ask a question or ...

Speaker 1

Um, would you prefer then on the front page to have a phone number to call where-

Speaker 2

If it was- if it's kind of an emergency.

Speaker 1

Um, well, it could be the- the same, uh, like you would answer the same questions with the nurse or- or whoever's on the phone.

Speaker 2

Mm-hmm.

Speaker 1

But ...

Speaker 2

So my question is like if ... what if it is like of a more em- urgent need to be seen? Is this, um, to make an appointment or for-

Speaker 1

It's to make an appointment.

Speaker 2

For the call back.

Speaker 1

Your- your goal was to make an appointment.

Speaker 2

Yeah, to make an appointment, I would ... I would, uh, and maybe it's just saying within the next week but maybe it really is within the day.

Speaker 1

It could be faster.

Speaker 2

But I do feel like a call back because the- the email's gonna come and if I'm trying to make an appointment, I need to say like I'm available on Tuesdays and Thursdays only or I need it, like, as soon as possible, what's your next opening? And then-

Speaker 1

The email-

Speaker 2

... there's the communication.

Speaker 1

The email will-

Speaker 2

Would have opt-

Speaker 1

... give you a link-

Speaker 2

... options and then you s-

Speaker 1

... to log on so you can see the appointment options.

Speaker 2

Okay. So, why can it not just be in this from the first thing? You have to be like kind of verified that it's bad enough, that my headache and my fever are bad enough to make an appointment.

Speaker 1

Um, they have to screen it first.

Speaker 2

Okay.

Speaker 1

Um.

Speaker 2

So this is the screening just to be able to get the link to make the appointment?

Speaker 1

Yeah. For whatever reason-

Speaker 2

Mm-hmm.

Speaker 1

... the Finnish Student Health Service doesn't let you just make appointments-

Speaker 2

Okay.

Speaker 1

... without having gone through screening.

Speaker 2

Well, then, then maybe I don't need the call back as long as that email is then allowing me to make the appointment as soon as possible.

Speaker 1

Yes, yeah.

Speaker 2

Yeah. So maybe that information could be in there. We'll contact you with the email attached to your account within the next ... so we can make a ... not making a, um, appointment. Um. Maybe there's a way of saying instead of within the next week, maybe it's saying as soon as poss- you know, like-

Speaker 1

But, uh-

Speaker 2

I know you need to say you have a wee-

Speaker 1

... you wouldn't want a more accurate, like, window so that you know if it's been a week then something's wrong.

Speaker 2

How ... so what happen- what needs to happen between me submitting this and someone sending the email? Someone has to verify the cause? The- the need?

Speaker 1

S- some medical professional.

Speaker 2

Has to verify the need for m- giving me an appointment?

Speaker 1

Yes. I don't know because I don't work for the Finnish Student Health Service but I do know that the way it works i- is that you have to be screened first.

Speaker 2

Okay. So if this is the screening, I'm filling it out.

Speaker 1

Yeah.

Speaker 2

If this is the procedure, then it is what it is. I do have to just-

Speaker 1

Yeah.

Speaker 2

... wait until I get-

Speaker 1

Yeah.

Speaker 2

... um, check my inbox for updates.

Speaker 1

But you might ... would you have preferred to have a phone call and answer the same questions with even just a receptionist and maybe have some opportunity to ask more questions?

Speaker 2

So, um, but the problem is I'm not the student demographic as much. Students are probably like, they're, they could be in class.

Speaker 1

Imagine you're a student. (laughs)

Speaker 2

Yeah. So if I'm a student, then this is probably best. I could be in class or it could be off hours and I could not meet a person and I can just do this-

Speaker 1

Yeah.

Speaker 2

... and go, "I am feeling these symptoms and they're not going away."

Speaker 1

Yeah.

Speaker 2

"Send me the email, let me make an appointment." So that's probably best.

Speaker 1

Okay. Thank you. Um, how did you feel about the progress indicators, like along the top and also the number system along the bottom?

Speaker 2

That was good.

Speaker 1

Were they helpful?

Speaker 2

Mm-hmm.

Speaker 1

Were they confusing?

Speaker 2

I like the fonts. I like the clarity, the size, the color. It's all very ...

Speaker 1

Was it intuitive? Did you know what the indicator along the top was- was describing?

Speaker 2

Mm-hmm. Yes, that's clear.

Speaker 1

What did- what did you think when you got to the first question?

Speaker 2

I'm putting questions. Um, I think it's great to just have that unlimited. It's not like in 30 characters, right? Like this ... I could just write.

Speaker 1

But I mean when- when you got here-

Speaker 2

Mm-hmm.

Speaker 1

... and you read these along the top, um, what did you think they were? Like what did they cor- the headings on the top, the sections, what did you think that they corresponded to?

Speaker 2

My current condition, previous care and then my coverage and then confirm it. I think that's good.

Speaker 1

What did you think it meant?

Speaker 2

What's one out of five? Is this ... are there- are there four?

Speaker 1

That's- that's my question. Like is that confusing to you?

Speaker 2

'Cause it says w- this is one out of five but really there's-

Speaker 1

There's five questions. There's four sections.

Speaker 2

Oh, five questions in the first section. Gotcha.

Speaker 1

And so the first section has-

Speaker 2

That's five.

Speaker 1

... three questions. Previous care is one question and health coverage is one question.

Speaker 2

And then ... okay.

Speaker 1

So it goes two, three-

Speaker 2

Yeah, I see now.

Speaker 1

... four-

Speaker 2

So that tells you, like it's just-

Speaker 1

... five.

Speaker 2

... whatever percentage or how far you're along in the thing.

Speaker 1

It's, it's, uh, what section you're in.

Speaker 2

Yeah. Yeah. Well, that's good.

Speaker 1

But it wasn't-

Speaker 2

Yeah.

Speaker 1

... intuitive or was it confusing?

Speaker 2

I mean, if you're, if I'm looking at it like that, current condition, um, I think that's the only way to do it. I think that's, that's good.

Speaker 1

It couldn't be made clearer?

Speaker 2

Um, you could not have page numbers, and they could only be following this. Um... (clears throat)

Speaker 1

But, uh-

Speaker 2

But then it would be unknown how long this thing is gonna take.

Speaker 1

Yeah.

Speaker 2

So page numbers are good. Um, could you put... Did you say two were on this?

Speaker 1

Three, one, one, the end.

Speaker 2

Then maybe, um... Can you put all three on the one, on one page and make it where all three pa- three questions, one question, one ques- so that each four thing is one page?

Speaker 1

Uh, you could do that. Would you think that's better?

Speaker 2

Let me see. Go to one, go to the second page. And so the bars would just be smaller. Um, and then this one box could be at the bottom of all three questions. So what's the third, what's the third question? Yeah, so, um, so you could do question one, two, three, and then have additional details block, right?

Speaker 1

Mm-hmm.

Speaker 2

Where do you go to previous? Describe your condition. Okay.

Speaker 1

Um, how did you feel about the input methods for each question? Do you think that they were intuitive or limited in any way?

Speaker 2

Um, I didn't feel like they were limiting. I f- felt like there was the full range that I would need. And then I have, could type details if, if I was like two days but severe, you know?

Speaker 1

Yeah. Um...

Speaker 2

Um, it was great to have a- at least the ability to type in more than standard. It's not like, it's not like, "Do you have a headache? Do you have a cold? Do you have a whatever?"

Speaker 1

Yeah.

Speaker 2

So it's good to ha- type in exactly what it is. And so, I think there was plenty of room to explain for someone to at least make an ap- have enough information to send me the thing for me to make the appointment with-

Speaker 1

Yeah.

Speaker 2

... with the person that they see fit or whatever.

Speaker 1

Mm-hmm.

Speaker 2

So the only thing then would maybe... if you're wanting this to flow properly with the amount of questions they're seeing. Um, the... Wait, wait, wait. Go to the confirmation page. "I regret my questionnaire

Speaker 3

One more person. We'll contact you today and we'll attach your account within the next week." Yeah.

Speaker 1

Um, do you have any other general feedback about the experience?

Speaker 2

Um, I really do like the simple of the font. The, the images were good. The two, the person and the telescope at the end. I like the gray boxes. It's very clear and clean. There's not too much on one page, and it's short and it flows. I think it looks really good. So the only problem would be how the one to five and the four aren't necessarily lining up. So I don't know if you wanna consider that input. It might not be improvement, you know, based on your opinion. But, uh, "Please describe your current condition." (computer dinging) Let's hope those are fireworks for New Year's. Okay. "Please describe your current condition." So, I don't know if, if that would be like, you know... And then you'd have to make these boxes smaller unless there was a way to say, you know, "Current condition, one, two, three," or...

Speaker 1

You don't have to solve it. (laughs)

Speaker 2

Oh, okay. Yeah. So my only thing was... I think the pages all look good. It's, it's not too little information where I'm like d- not feeling like I was able to express, like, m- my needs-

Speaker 1

Yeah. Yeah.

Speaker 2

... my... And then, it's simple enough. It's not like I had to fill out, like, way too much information just to try to get appointment.

Speaker 1

Yeah.

Speaker 2

Um, does it... Um,already. So it doesn't need to know, you don't have to input your healthcare 'cause it just, if you're a student, then it's covered?

Speaker 1

Um, yes.

Speaker 2

Is that the only question? Okay. Th- that is good. If that's the qui- only question that needs to be answered, fine, unless you need to, like, enter your healthcare coverage.

Speaker 1

I think that that will be-

Speaker 2

Everyone has the same.

Speaker 1

... suited later or f-

Speaker 2

Okay.

Speaker 1

... solved later. Yeah.

Speaker 2

Okay. And then... That's important 'cause then it could be like the urgency of the appointment, right? Um...

Speaker 1

You don't have to also-

Speaker 2

Oh.

Speaker 1

... critique the questions themselves.

Speaker 2

Okay.

Speaker 1

It's, this is more a study of the-

Speaker 2

Flow?

Speaker 1

... the flow, the user experience.

Speaker 2

I really like the font. I really like the simplicity. I, I really like the layout and the simplicity and the choice-

Speaker 1

Mm-hmm.

Speaker 2

... the simple choices, the couple boxes to type in, couple quick select-

Speaker 1

Yeah.

Speaker 2

... the severity, and then yes, send me an email. I like it.

Speaker 1

Okay. Thank you for your participation.

Speaker 2

You're welcome.